The Concept of Prevention

THE PUBLIC HEALTH ORIENTATION

Community psychology has been inspired by the example of public health medicine in the conquest of disease. While clinical medicine focuses on the treatment of the sick individual, public health medicine is aimed at reducing the in-

cidence and prevalence of disease in the population.* Much of the success in combating major diseases has occurred through public health means; in comparison, the gains of clinical medicine are minor (Dubos, 1961). Logically, if one could avert disease rather than cure it when it occurs, there would be greater relief from human suffering. Programs of inoculation, sanitation, health education, and various kinds of environmental interventions have effectively removed the threat of e.g., typhoid, malaria, and polio, while reducing infant mortality and increasing longevity greatly. For many mental health workers, public health provides a frame of reference as well as an inspiration for their efforts (e.g., Eisenberg, 1962; Caplan, 1964).

Public health intervention may occur at three distinct points. (1) The state of the individual can be so altered as to increase his *resistance* to disease agents. Examples are readily available for those conditions for which antibodies have been developed which provide immunization against a disease; (2) The probability of *contact* with the disease agent is reduced or its virulence is lowered. Control of contagious diseases involves quarantine, wearing of face masks, disinfecting exposed tissue, and the like; (3) The *environment* conducive to the development of the disease agent can be changed in order to destroy the disease agent. The classic illustration involves the draining of swamps in which the anopheles mosquito, carrier of malaria, is propagated. Ideally, the etiology of the disease should be known in order to develop rational and effective preventive interventions. In fact, however, simple empirical observations, sometimes even involving fallacious hypotheses, have led to effective treatments. Thus, Jenner noticed that people who had cowpox seemed to be immune to smallpox, a much worse disease. By giving people small doses of cowpox he developed his vaccine for smallpox, long before the cause or mechanism of the disease was understood.

Public health workers distinguish between primary, secondary, and tertiary prevention. *Primary prevention* is aimed at reducing the possibility of the disease in a susceptible population (population-at-risk), as might happen with the development of a new vaccine. The goal is to prevent people from becoming ill in the first instance. *Secondary prevention* attempts to reduce the duration, prevalence, or contagiousness of the disease in those already ill. Emphasis is on early detection and early treatment. Urging people to have periodic X-rays in order to note at the earliest moment developing tuberculosis or cancer, or the admonition that one should see a dentist twice a year, are familiar instances of secondary prevention programs. *Tertiary prevention* seeks to reduce the consequences of a disease among those who have already had it. This might involve, for example, providing an amputee with an artificial limb, teaching him new job skills and helping him overcome a sense of inferiority so that he can resume a productive life. Although analogies are risky, and boundaries are indistinct, community psychologists have used these terms to characterize preventive approaches in the realm of mental health.

* The terms "incidence" and "prevalence" are widely used in public health to describe two related but independent concepts. Incidence refers to the number of people who *become* ill in a particular time period; prevalence describes the number of people who *are* ill at that time. Thus, the incidence of psychosis might be measured by the number of admissions to psychiatric hospitals. Prevalence would be indicated by the number of people in hospitals. The difference between the two rates is largely a function of the duration of the disease. If the disease is one from which people recover very quickly, then incidence and prevalence are very close.

PRIMARY PREVENTION

Many in the mental health fields have been understandably skeptical about the possibility of primary prevention. The Joint Commission on Mental Health and Mental Illness concluded simply: "... primary prevention of mental illness has remained largely an article of scientific faith rather than an applicable scientific truth" (*Action for Mental Health*, 1961, p. 70). In reply René Dubos said at a meeting of the National Association for Mental Health in 1962 (quoted by Sanford, 1965):

> The Final Report of the Joint Commission on Mental Health and Mental Illness states that humane treatment and rehabilitation of the mentally ill is the great unfinished business of the mental health movement. But the great unfinished business really is to do something about the social, psychological and other circumstances leading to this condition [p. 1378].

That is the challenge of primary prevention, which as Dubos indicates elsewhere "must be undertaken since no major disease in the history of mankind has ever been conquered by therapeutic and rehabilitative methods alone, but ultimately only through prevention" (Dubos, 1961). At this point, our knowledge of the causation of emotional disorders is both limited and gross, as are our techniques and power to effect change, particularly at the broader social levels. However, guiding ideas have been proposed for developing methods oriented to prevention.

In his conceptual model of "preventive psychiatry," Caplan (1964) starts with the basic assumption that all human beings need "supplies" appropriate to their level of development. Deficiencies in these supplies can lead to psychological disorders just as insufficient food or the wrong foods can result in malnutrition. Supplies of three kinds are needed for healthy psychological development, which Caplan labels physical, psychosocial, and sociocultural. Physical supplies include food, shelter, protection from harm, and sensory stimulation, among others. Psychosocial supplies are those received through interaction with others, and include emotional and intellectual stimulation, love and affection, participation in satisfying social encounters, and the like. If the individual does not hold the respect of others or if they manipulate him toward their own ends, emotional disorders may result. Finally, the term sociocultural supplies is used to characterize those social forces which determine the status of the individual and the expectations of others for him. In advantaged social groups, individuals have greater input of such supplies, which predicts healthier personality development. In disadvantaged groups, or in times of rapid social change, traditional social supports may be undermined and psychological distress is more likely.

Another factor of importance in Caplan's scheme is the central role of crisis resolution. Life crises, whether accidental or developmental, both endanger existing adaptation and can lead to mental disorders or provide opportunity for further growth and development. Hence, helping individuals overcome crisis situations is a critical part of primary intervention. Concepts and methods of "crisis intervention," deriving from Caplan's view, have figured importantly in community psychology programs, and will be discussed in some detail in the next chapter.

Both in order to provide the necessary supplies and to strengthen crisis coping abilities, Caplan notes that primary prevention efforts are possible both at the

level of "social action" and at that of "interpersonal action." The former consists mainly of efforts to influence legislative and social programs serving welfare, health, or educational needs in the larger society; interpersonal actions are intended to influence individuals, families, and community groups through more direct, face-to-face interventions. These involve not only helping with people in need (as in crisis intervention) but also indirect actions on their behalf through consultation with "community caretakers," such as teachers, doctors, and political leaders, who are important to the lives of many in the community. Because of its importance in community psychology, we will consider concepts and methods of consultation in more detail in the next chapter.

What kinds of things can be done to foster primary prevention? In general, both the potential interventions as well as the potential distress-producing factors can be conceptualized as residing at the level of the larger society, the community, the face-to-face group, or the individual.

In the Larger Society. Any social program which increases the quality of life, fosters education, social welfare, or improved medical care, or increases job opportunities, in principle provides needed physical, psychosocial, and sociocultural supplies. Thus, improved prenatal medical care for slum mothers should reduce the number of children born psychologically and neurologically defective (Eisenberg, 1962c). Similarly, urban renewal programs can provide a psychologically as well as physically healthier environment, if attention is given to the human issues, the needs for social contact, stimulation, and recreation as well as bricks-and-stone matters (Fried, 1964b). To this end, mental health specialists have consulted with urban planners in the design of new cities (e.g., Lemkau, 1969). Social programs intended to protect family integrity can have important consequences, for the family is a central source of psychosocial supplies for the growing child. Thus, legislation that would allow young mothers to work part-time, have adequate day-care facilities, or time off to spend with the child could importantly influence the well-being of both child and mother. Similarly, changes in welfare regulations that would have the effect of encouraging, rather than as now discouraging, both parents to live with a dependent child might also foster early development. At a broader level, sociocultural needs are served through reducing job and housing discrimination, increasing educational and job-training opportunities, and protecting civil rights. Educational programs such as "Headstart," intended to overcome educational handicaps in deprived children can lead to increased social competence and heightened self-esteem.

In this realm it is important to note, the direct power of psychologists to effect changes is distinctly limited, for they depend on political processes beyond their control. The greatest contribution of psychologists and other behavioral scientists is often through research and consultation, as occurred at the time of the Supreme Court ruling on school segregation. Groups of concerned psychologists and psychiatrists, affiliated with the Society for the Psychological Study of Social Issues and the Group for the Advancement of Psychiatry, assembled all available evidence on the effects of segregated schooling on white and black children. The upshot of their analyses was that segregation resulted in poorer education for both races. The Court used these findings in reaching its decision. Increasingly, behavioral scientists and mental health specialists are being called on to provide needed facts and to advise policy makers in many areas of social legislation.

In the Community. Many efforts of the psychologist in the community can be conceived as contributing to primary prevention. Community leaders are educated to the needs for physical, psychosocial, and sociocultural supplies in various segments of the population. Similarly, programs of mental health education are launched intending to alter public attitudes and instill values more favorable to positive mental health (see Chapter 18). Community psychologists consult with urban planners in the development of facilities for recreation and housing and with community caretakers at all levels to assure that their services maximally serve the psychological needs of the people. Thus, social agencies may be encouraged to better coordinate their services so that many of the problems which people have are not neglected. Consultation with important community agencies, such as the police, is undertaken so that their work can be reconceived in terms of serving psychosocial needs. In his project, Bard and his collaborators (e.g., 1967, 1969, 1973; Zacker and Bard, 1973) have demonstrated that community psychologists training and working with police teams can make them effective agents of crisis intervention with troubled families.

Much of the effort of community psychology has focused on altering the social processes of school systems in order to make them better environments for emotional growth (e.g., Sarason et al, 1966; Sarason, 1972; 1974). At their most ambitious these efforts attempt to change basic patterns of communication and organization and attitudes of teachers and administrators, to create a more growth-inspiring environment for school children. More modest efforts involve consultation with teachers to enable them to recognize and cope with emerging psychological problems of children. These efforts, more in the nature of secondary than primary prevention, are thought by many to represent a more productive direction for community psychology. Thus, Cowen et al (1973, p. 13) say "Although primary prevention in schools is an attractive ideal, it is both abstract and futuristic. Accordingly, there is need to develop theoretically less ambitious, but practically more feasible, approaches that will facilitate young children's school adjustment." Whatever their direction and rationale, however, school programs provide a major focus of community psychological activity, for it is in school that a major portion of psychological growth takes place. Any effort to make the educational setting more productive of personality development, as well as traditional learning, is to be applauded.

Finally, community psychologists work with community groups themselves to overcome feelings of apathy and powerlessness and increase their capacity for collective action so that they can better serve their own needs. Work in "community organization" which involves the identification and encouragement of leadership and the facilitation of group process and communication and similar processes is often akin to the work of organizational psychology in business, government, and other formal organizations and makes use of similar techniques.

The Family and Small Group. Character is formed and psychological problems develop primarily within the family which has the fundamental role in providing necessary psychosocial supplies. Intervention may involve parent-education programs, prenatal instruction, and various forms of family therapy and family-crisis intervention. Sometimes such programs can best be based in well-baby clinics or

other pediatric services to bring them to families who would not seek the help of a "psychiatric clinic," or they may be adjunctive to the school or even police or probation departments. The critical task is to help in redirecting parental efforts before serious problems emerge with their children.

Particularly in work with adolescents, there is increasing recognition of the peer group as a primary socializing agency. Consequently, programs directed toward the control of delinquency and drug abuse as well as less extreme problems of adolescent development have focused on the peer group. Some of these are found within existing institutions, for example, Boy Scouts or church groups, while others involve the development of essentially new social enterprises such as Project Community (Soskin, Ross, and Korchin, 1971). In this project, a membership organization was created for high-school aged disaffected youth in Berkeley which gave them their own "place" in which they could discover new modes of social interaction with peers and young adults, engage in self-exploratory and quasi-therapeutic group activities and in "workshops" in which they learn about themselves and facets of their world usually ignored by the schools. A gratifying consequence of participation was a drop in drug usage and apparent increases in social trust, motivation and a sense of purpose and direction in otherwise foundering young people.

The Individual. The main purpose of individual-centered interventions is to strengthen the person's ability to cope with life crises. They may involve either direct contact with the person in the crisis, which is similar to short-term, focused psychotherapy (see Chapter 18), or they may include consultation with caretakers, families, or friends who are directly involved with the person. This is conceived as a preventive measure by Caplan because he sees the time of crisis as a unique opportunity for fostering adaptive coping in the face of future stress. He also calls attention, however, to the fact that some crises occur in many lives at predictable points and therefore allow for anticipatory crisis intervention. Special attention should be given to such "developmental crises" as starting school, getting married, becoming pregnant, becoming physically ill, or retiring. Programs can be worked out for many people simultaneously who share these experiences and intervention can occur in such settings as college dormitories, prenatal clinics, divorce courts, personnel offices, and the like. By lessening the stress of the on-coming crisis, primary prevention is served.

SECONDARY PREVENTION

The aim of secondary prevention is to reduce the prevalence of emotional disorders through early diagnosis and prompt, effective treatment. The essential assumption is that incipient problems can be detected through early signs and that interventions at that point can head off greater problems in the future. Evidence for the effectiveness of early detection and treatment have been found in a number of realms (e.g., Caplan, 1961; Sanford, 1965; Eisenberg, 1962c, 1962d). Thus, it was found in the military services during the Korean War that treatment quickly given as close to the battle lines as possible significantly reduced the need for later treatment and hospitalization of psychiatric casualties (Glass, 1963). Such evidence, it might be argued, simply asserts that prompt recognition of a problem and early treatment is simply good clinical practice, and indeed it is.

What is distinctive about secondary prevention programs is that they involve (1) widespread screening testing of people who are not seeking help or may even be asymptomatic at the moment and (2) techniques for bringing them quickly to appropriate clinical services. The screening devices are usually both briefer and different from the techniques used in conventional clinical practice, of the kind discussed in Part III. The essential hope is that problems can be spotted before they become serious and when relatively less costly or intensive methods can be used to deal with them.

Schools provide a unique setting for secondary, like for primary, prevention activities. For many years, programs have been underway in different communities designed to identify early problems of school-aged children (e.g., Allinsmith and Goethals, 1962; Zax and Cowen, 1969; Glidewell and Swallow, 1968; M. Levine and Graziano, 1972; Cowen et al., 1973). These projects usually involve training teachers to recognize early signs of behavioral disturbance of the sort that might warrant referral to a mental health agency. Sometimes, interview or questionnaires are given to the parents. In addition, brief, quickly administered, and easily scored screening tests of considerable utility have been developed (Bower, 1969; Cowen et al., 1973).

However successful screening programs might be, they achieve little if appropriate intervention does not then follow. The distressed individual, or his family or community caretakers, should be prepared to seek appropriate help. Secondary prevention, therefore, necessarily involves public information programs toward three distinct though related purposes: first, they should inform about the signs of incipient conditions; second, they should give information about available resources, what is available, where, at what cost, etc.; third, they should attempt to combat prejudices against seeking help, misconceptions about what mental health professionals do, and the like. It was found in a national survey that over two thirds of those who felt they needed psychological help sought the services of a clergyman or a physician rather than of a mental health clinician (Gurin, Veroff, and Feld, 1960). Perhaps the choice was not inappropriate and they found help for their problems, but it may be that earlier contact with a clinician would have served better. One purpose of education is to clarify the circumstances under which one should seek clinical psychological help rather than spiritual or medical assistance, and such education should as much be directed toward clergymen, physicians, and other caretakers as to the public itself. In most communities, there is at the least great confusion as to the variety of helping agencies available and great resistance to using them.

If early detection has been effective, and people want and know where to seek early help, helping agencies must then be prepared to give quick and appropriate service, if the overall program of secondary prevention is to be successful. Too often, clinics are tied up in complex procedures and red tape which greatly delays treatment or even completely discourages prospective clients. To be of greatest help, services should be readily available, close to the patient's home, and well publicized. The development of twenty-four-hour "walk-in clinics" and emergency services, as part of the community mental health center, or of "hotlines" and other rapid referral services has helped greatly in making care immediately available. Ideally, both facets of secondary prevention—early detection and immediate intervention should occur within the same setting, so that the process can flow smoothly and continuously. This is possible and has been done in such

organizations as schools, military units, and colleges, but it is less easy in other settings.

TERTIARY PREVENTION

The task of tertiary prevention is to limit the aftereffects of mental illness and hospitalization and to restore patients to effective functioning in the community. Just as secondary prevention focuses on people in danger of becoming emotionally disturbed, tertiary prevention is concerned with the one who has *been* disturbed and is now recovered or recovering. There are three major areas of emphasis in tertiary prevention.

Rehabilitation. Rehabilitative services work to restore the former patient's self-confidence and his social and vocational competence. They aim to make him a self-respecting and respectworthy citizen of the community, able to resume family, work, and social roles or take on new ones. Rehabilitation often includes therapeutic counseling, aimed at strengthening the patient's readiness to confront social challenges and possible social rebuff. At the same time, there is specific training in job-related skills that can prepare the former patient for a new occupation or to be better prepared to resume an old one. Rehabilitation centers serve not only former psychiatric hospital patients but also the somewhat parallel problems of the blind, deaf, amputees, and others recovering from severe illnesses. For such efforts to be meaningful and effective, they must be integrated with earlier hospital care. In the past, large, distant, custodial institutions made little contact with rehabilitative agencies. As mental health services become more community-oriented, within-hospital treatment and after-hospital rehabilitation should become more integrated and continuous. The psychotherapy and occupational therapy in the hospital should be geared to the problems the patient will confront when he leaves the hospital.

Changing Community Attitudes. One of the major problems faced by the patient leaving a psychiatric hospital is the distrust, rejection, and stigmatization he may face in the community. Unlike the medical patient, the former psychiatric patient, like recently released prisoners or former drug addicts, is likely to be received with distrust and fear. People too readily assume that the former patient can become dangerous to himself and others and they are reluctant to admit him to their homes or offices. Patients find it hard to get good jobs, or in general, to be treated with self-respect. A major purpose of mental health education, as we will discuss in Chapter 18, is to develop more rational and humane attitudes toward those who have suffered severe mental disturbances.

Partial Hospitalization and Other Alternatives to Hospital Care. All too often, patients continue to stay in hospitals simply because they have no where else to go and long after there is any clinical justification. They may have no families or their families are unwilling to put up with them; they may lack the courage or skills to face the competitive outside world. For many, leaving the hospital means drifting into a skid-row existence of slow deterioration. Ideally, intermediate institutions are needed which allow the patient to make the transition to community life in limited steps. Until now, the choice has been basically between inpatient

hospitalization or independent living, perhaps with some outpatient therapeutic and rehabilitative service. Transitional institutions, such as night or day hospitals and halfway houses or hostels for those needing partial but not full hospital care are being developed and are effective substitutes for psychiatric hospitalization. Day, night, and weekend hospitals are alternate modes of partial hospitalization which give the patient a retreat or home and/or a place for therapeutic activities while at the same time allowing him to work or go to school, maintain contact with family and friends, and otherwise participate more fully in the life of the community. As a historic aside, it is interesting to note that the first day hospital for mental patients was started in Moscow in 1934, though apparently more for practical than therapeutic reasons (B. M. Kramer, 1962). Anyone with administrative interests realizes that the same physical plant and staff can serve a larger number of people if each one uses it on a partial basis. The better justification for partial hospitalization is that it does not separate the patient from his community.

Concern with posthospitalization rehabilitation for the mentally ill in the United States is relatively recent. There were few facilities before 1950. By contrast, in Europe and particularly in England, there is a much longer and more developed interest in psychiatric rehabilitation. A notable example is the so-called Marlborough experiment under the direction of Joshua Bierer (1964) which has inspired many American community mental health workers.

In 1938, Bierer formed a "therapeutic social club" which was followed in short order by a day hospital, a night hospital, an aftercare rehabilitation center, a self-governed community hostel, "neurotics nomine," and a weekend hospital. Together, they became a program of comprehensive care. "The primary goal of the experiment was to enable the patient to change his role from that of a passive object of treatment, etc., and to become, instead, an active participant and collaborator in the treatment process" (Bierer, 1964, p. 223). The therapeutic social club started within the hospital and involved membership dues, special rooms, and recreational facilities. Meetings were held, including some with the hospital staff, which developed social activities, discussion groups, games and painting groups, as well as group therapeutic and psychodramatic activities. Membership included people of all types, "cockneys and patients with very cultured backgrounds . . . brilliant scientists and . . . mentally retarded patients." In time, as patients were discharged, the clubs continued outside of the hospital and, as reported in 1964, some had been in existence for over twenty years. They apparently became focuses on group identification and sources of social support for their members long after they needed hospital care.

The day, night, and weekend hospital services developed by the Marlborough workers made earlier discharge possible from the full-time hospital services. The aftercare rehabilitation center they developed was similar in type to the "sheltered workshops" used in physical rehabilitation, in which people develop skills in a controlled environment that makes less demands of them than an ordinary shop or factory. The self-governed community hostel represented still another mechanism for serving a basic need of former patients, namely, decent and congenial housing within their means. A house was rented and the patients themselves established a living unit which they supported and ran by their own efforts. The hostel was a source of pride and accomplishment, which fostered cohesiveness in the group. Contrast this with the all-too-common experience of patients who move from the back wards of hospitals to the back rooms of anonymous slum ho-

tels. Providing adequate halfway houses and other transitional settings for patients not yet ready to reenter conventional social life is a major challenge to the field (Raush and Raush, 1968).

Neurotics Nomine, developed and sponsored by Bierer in the Marlborough experiment, was another type of patient alumni organization. The patients in these groups took on themselves the task of helping others in mental hospitals, by visiting, counseling, and giving emotional and social support to fellow sufferers. More elaborate forms of the same sort of organization can be seen in groups such as Alcoholics Anonymous, Recovery, Inc., and Synanon in the United States. These can be very effective ways of maintaining the social adjustment and self-respect of their members, while being of distinct help to others in distress.